
Dysphagia FAQs

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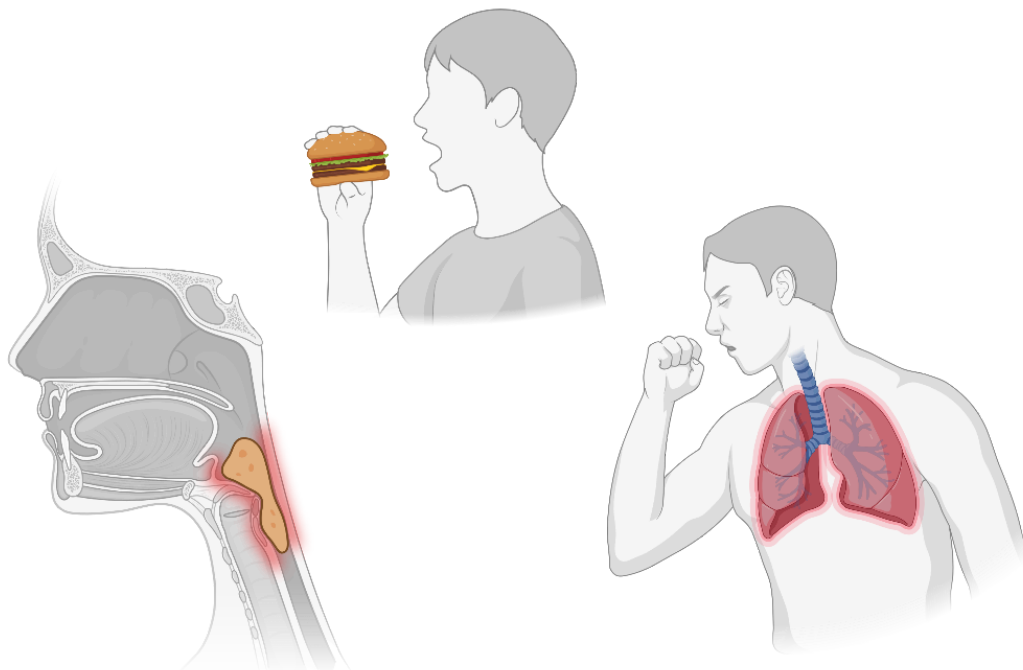
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It can be frustrating that something as natural as swallowing can be impacted by Parkinson's. It can also be overwhelming to learn about all of the ways that disordered swallowing can impact your health and how we can manage it.

Terms defined in this handout are:

Dysphagia, Oral Hygiene, Physical Mobility, Aspiration, Modified Diets, and Swallow Exercises.

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Q:

WHAT IS "DYSPHAGIA"?

People with Parkinson's disease are at risk for dysphagia. Dysphagia is a swallowing disorder affecting how well the tongue and upper throat muscles squeeze food, liquid, or saliva down into the esophagus. Try filling out a questionnaire called the '[EAT-10](#)' to see if you have symptoms of dysphagia, and talk to your doctor if you have concerns.

A:

Acute or chronic dysphagia can sometimes result in 'aspiration' of food or liquid into the lungs. This means that small amounts of certain swallowed items may be 'going down the wrong pipe'. Aspiration can make you cough, but it is important to know that *you may not always feel it*. If you have concerns about your swallowing, you will likely have an exam with a speech-language pathologist (SLP) called a modified barium swallow study, or a fiberoptic endoscopic evaluation of swallowing.

When someone has dysphagia because of Parkinson's disease, then aspiration can occur rarely, sometimes, at certain times of the day, or always. Just like your other Parkinson's symptoms, it can vary. Sometimes, aspiration can lead to pneumonia - especially in certain situations (e.g., the aspirated material is food instead of liquid, is highly acidic, has a high bacterial load, or if the body is very weak).

Q:

WHY DOES "ORAL HYGIENE" MATTER FOR PEOPLE AT RISK FOR DYSPHAGIA?

Oral hygiene does not alleviate the swallowing disorder itself, but it helps protect against pneumonia for people who have dysphagia or sometimes aspirate.

Good oral hygiene means toothbrushing at least twice a day, flossing, using mouthwash, and regularly visiting the dentist to take care of cavities and tooth replacements. Also, using oral moisturizers can be good protection for your teeth if you suffer from dry mouth!

A:

Oral hygiene helps reduce the bacterial load of your mouth, which in turn reduces the bacterial load of what you swallow. Usually, stomach acid can handle this bacterial load - but if something 'goes down the wrong pipe' and aspiration occurs, our lungs CANNOT handle bacteria very well.

During a sickness or hospital stay, keeping your mouth clean with toothbrushing and mouthwash is especially important - these are times when your immune system is exhausted. Even if you don't regularly aspirate when you are healthy, it's possible that a temporary decline can happen during a hospitalization.

Q:

WHY DOES "PHYSICAL MOBILITY" MATTER IN PEOPLE WITH DYSPHAGIA?

A:

Staying as active as you safely can and keeping your immune system strong is very individualized. If you are medically able, engage in physical therapy or whole-body exercise. This is important for anybody with Parkinson's, and even more so if that person has dysphagia.

Even just focusing on safely moving around your house every day, rather than sitting, can help your lungs stay clear and healthy. If someone has dysphagia and aspirates, then staying in one position all the time can cause the aspirated material to sit stagnant in the lungs and grow more bacteria.

Q:

WHAT IS A "MODIFIED DIET"?

A modified diet can include easier-to-eat textures or types of food, and thickened liquids.

A:

Sometimes, a modified diet will be prescribed as a way to help someone compensate if their dysphagia is more severe. Usually, thickened liquids are for short-term use and only upon recommendation of an SLP. An SLP decides whether to recommend a modified diet depending on many things, such as the preference of the patient, risk of malnutrition and dehydration, risk of pneumonia, or cognitive status. Swallowing pills can also be tricky, so be sure to speak with a speech-language pathologist for individualized tips!

Look for recipes that help you balance appropriate textures, appetizing flavors, and nutritional content. Smoothies can sometimes be a great addition to your routine.

If you have trouble with some of your favorite meals or drinks, have discussions with a speech-language pathologist to understand your level of risk and brainstorm your options. There are many ways to manage your dysphagia - modified diets are not the only way!

Pro-tips: If thickened liquids are recommended to you, gel-based thickeners taste better when the liquid is very cold (put it in the fridge!), and thickened water tastes better when flavoring is added.

Q:

WHAT ARE "SWALLOW EXERCISES"?

There are many rehabilitative exercises that a speech-language pathologist can prescribe for the muscles of your mouth, airway, and throat.

A:

Exercise for these tiny muscles works the same as exercise for any other muscle of the body. Depending on which of your mouth or throat muscles are impaired (determined by an SLP assessment), you may do repetitions against resistance or you may learn to practice certain dynamic movements such as an 'Effortful' swallow.

There is emerging research on multiple different devices to help facilitate certain types of exercise; one example is using a pressure-threshold respiratory strength training device for people with weakness in their upper airway or respiratory support.

More resources

Podcast ▾

How oral hygiene helps people with Parkinson's:
[A Parkinson's Foundation Podcast \(Episode 47\)](#)

PDF ▾

Use the '[EAT-10](#)' questionnaire to see if you have dysphagia symptoms.

PDF ▾

[National Foundation of Swallowing Disorders](#)

PDF ▾

[Parkinson's Foundation resources on speech and swallowing](#)
(Found at [Parkinson's Foundation resource library](#))

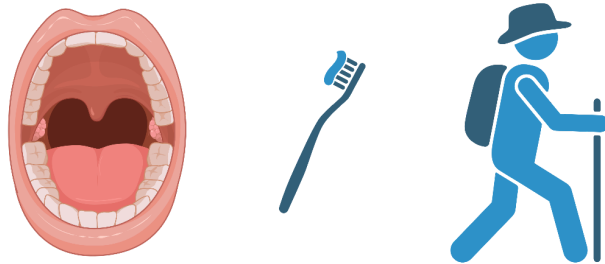
Book ▾

A dysphagia cookbook:
[Modern Dysphagia Cooking \(on Amazon\)](#)

Website ▾

A way to find speech-language pathologists in your area:
[ASHA Pro-Find](#)

Things like oral hygiene and physical mobility work best as preventative, regular habits.



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However, if you are actively experiencing symptoms of dysphagia, it is strongly recommended to *let your doctor know* and get an individualized treatment plan with a speech-language pathologist.